BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

Check	Before starting	First infusion	Between infusions	Course review	After treatment
Blood count, kidney, liver and glucose	Baseline blood tests	Results reviewed before dosing	Before every 3-week cycle	Confirm cause of abnormal tests; clinician decides hold, restart or stop	Follow unresolved abnormalities
Thyroid and other hormones	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length	Confirm metastatic or adjuvant plan	Agree how benefit will be assessed	Scans and review on your team’s schedule	Adjuvant: up to 12 months; metastatic: individual 24-month review	Agree surveillance or next treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00455 v16 · official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab 200 mg · every 3 weeks · lung cancer: treatment and checks

Lung cancer only: first-line metastatic NSCLC or adjuvant NSCLC after surgery and platinum chemotherapy

Pembrolizumab 200 mg is given as a drip over about 30 minutes on Day 1 every 3 weeks (21 days). The hospital visit is longer because of checks and preparation. After complete surgery and platinum chemotherapy, the adjuvant course lasts up to 12 months. For metastatic cancer, treatment continues while it remains helpful and tolerable; the team may consider stopping after 24 months of sustained response without progression, or continuing when appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Between infusions	Course review	After treatment
Treatment NCCP 00455	Confirm metastatic or adjuvant indication	200 mg IV over about 30 minutes	Day1 every 3 weeks while prescribed	Adjuvant maximum 12 months; metastatic individual review	Follow-up/surveillance or next treatment
Blood count, kidney, liver and glucose NCCP 00455	Baseline blood tests	Results reviewed before dosing	Before every 3-week cycle	Confirm cause of abnormal tests; clinician decides hold, restart or stop	Follow unresolved abnormalities
Thyroid and other hormones NCCP 00455 · SMPC	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects NCCP 00455 · SMPC	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length NCCP 00455	Confirm metastatic or adjuvant plan	Agree how benefit will be assessed	Scans and review on your team’s schedule	Adjuvant: up to 12 months; metastatic: individual 24-month review	Agree surveillance or next treatment

What each check is for

1 Before first dose

Before starting

Confirm the indication and baseline safety

Ask / act: Confirm results, access, consent discussion, alert card and responsible teams.

Sources: NCCP 00455

2 Course-length decision

At planned course review

Metastatic and adjuvant durations differ

Ask / act: Document whether treatment ends, pauses or continues, and how follow-up is arranged.

Sources: NCCP 00455

3 After the last dose

After treatment

Immune effects and hormone needs can continue

Ask / act: Provide the monitoring and urgent-contact plan.

Sources: NCCP 00455 · SMPC

4 Abnormal liver or kidney results

Whenever a result changes

A blood result alone does not establish immune inflammation

Ask / act: Contact the treating team and confirm the investigation and treatment plan.

Sources: NCCP 00455 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.26